

pharmaniaga®

— ActiMol Suspension 250mg/5ml —

PRODUCT DESCRIPTION

An orange suspension with a mentholated sweet and sour orange taste.

COMPOSITION

Each 5ml contains:

- Active Ingredient: Paracetamol 250 mg
- Preservatives: Methylparaben 9 mg and Propylparaben 1 mg

PHARMACODYNAMICS

ATC Code: N02 BE01.

Pharmacotherapeutic group: Other Analgesics and Antipyretics (Anilides).

Mechanism of Action

Paracetamol is an analgesic and antipyretic. Its mechanism of action is believed to include inhibition of prostaglandin synthesis, primarily within the central nervous system.

Pharmacodynamic effects

The lack of peripheral prostaglandin inhibition confers important pharmacological properties such as the maintenance of the protective prostaglandins within the gastrointestinal tract. Paracetamol is, therefore, particularly suitable for: patients with a history of disease or patients taking concomitant medication, where peripheral prostaglandin inhibition would be undesirable (such as, for example, those with a history of gastrointestinal bleeding or the elderly).

PHARMACOKINETICS

Absorption

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract.

Distribution

Binding to plasma proteins is minimal at therapeutic concentrations.

Metabolism

Paracetamol is metabolized in the liver and excreted in the urine mainly as glucuronide and sulfate conjugates.

Elimination

Less than 5% is excreted as unchanged paracetamol.

INDICATIONS

ActiMol Suspension 250mg/5ml is used for the relief of:

- Fever
- Headache and symptoms of cold and flu
- Toothache and discomfort of teething
- Fever after vaccination

RECOMMENDED DOSAGE

Using the body weight of your child, you can calculate the dose of Actimol your child needs. The calculation is based on recommended dosage of 15 mg paracetamol.

For ActiMol Suspension 250mg/5ml, Dose=

$$\frac{(BW \text{ (kg)} \times 15\text{mg} \times 5\text{ml})}{250\text{mg}}$$

Example of dose by weight:

WEIGHT (kg)	DOSE (ml)
12 - 13 kg	3 ml
14 - 16 kg	4 ml
17 - 19 kg	5 ml
20 - 23 kg	6 ml
24 - 26 kg	7 ml
27 - 29 kg	8 ml
30 - 33 kg	9 ml
34 - 39 kg	10 ml
40 - 42 kg	12 ml

Do not take more than 4 times within 24 hours and never exceed the recommended dose unless prescribed by a doctor or pharmacist.

Not recommended in children under 1 month.

For children under 3 months: If fever persists for more than 24 hours (4 doses) seek medical advice. This is to ensure that fever that may be due to a serious infection is quickly diagnosed.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Hypersensitivity to paracetamol or any of the other ingredients/components of the product. Severe and active hepatic impairment.

ADVERSE EFFECTS/UNDESIRABLE EFFECTS

System Organ Class Frequency	Significant	Rare	Potentially Fatal	Not known	Prolonged usage or overdose
Gastrointestinal disorders				Nausea, Vomiting, Constipation	Acute pancreatitis
Nervous system disorders				Headache	
Psychiatric disorders				Insomnia	
Skin and subcutaneous tissue	Angioedema			Hypersensitivity reactions including skin rashes, Erythema, Flushing, Pruritus, Stevens Johnson syndrome, Toxic epidermal necrolysis	
Blood and lymphatic disorders	Thrombocytopenia, Leukopenia, Neutropenia, Pancytopenia, Methaemoglobinemia, Agranulocytosis				
Renal and urinary disorders			Acute renal tubular necrosis		Nephrotoxicity
Cardiac disorders		Hypotension, Tachycardia			
Hepatobiliary disorders			Hepatotoxicity		Hepatic necrosis

WARNINGS AND PRECAUTIONS

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.

Allergy alert:

Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of serious conditions. If these reactions occur, stop using and seek medical assistance right away.

- Keep out of reach of children.
- Do not take if allergic to paracetamol.
- Patients should contact their healthcare provider if symptoms persist (if the pain lasts for more than 10 days, if there is redness or if a fever lasts more than 3 days).
- Paracetamol should be given with care to patients with impaired kidney or liver function.

- Large doses should be avoided in patients with hepatic impairment. Paracetamol overdose may harm the liver.
- Do not exceed recommended dose. Leave at least 4 hours between doses.
- Immediate medical advice should be sought in the event of an overdose, even if the child seems well, because of the risk of delayed serious liver damage.
- Paracetamol provides symptomatic relief only, additional therapy to treat the cause of the pain or fever should be instituted when necessary.

EFFECTS ON THE ABILITY TO DRIVE AND USE MACHINES

It is unlikely to impair a patient's ability to drive or use machinery.

INTERACTION WITH OTHER MEDICAMENTS

The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with an increased risk of bleeding; occasional doses have no significant effect.

The hepatotoxicity of paracetamol, particularly after overdose, may be increased by drugs that induce liver microsomal enzymes such as barbiturates, tricyclic antidepressants and alcohol.

Chronic alcohol intake can increase the hepatotoxicity of paracetamol overdose and may have contributed to acute pancreatitis reported in one patient who had taken an overdose of paracetamol. Acute alcohol intake may diminish an individual's ability to metabolize large doses of paracetamol, the plasma half-life of which can be prolonged.

The speed of absorption of paracetamol may be increased by domperidone or metoprolol and absorption reduced by cholestyramine.

Antivirals: Regular use of Paracetamol possibly reduces the metabolism of Zidovudine (increased risk of neutropenia).

The use of drugs that induce hepatic microsomal enzymes such as anticonvulsants and oral steroid contraceptives may increase the extent of metabolism of paracetamol resulting in reduced plasma concentrations of the drug and a faster elimination rate.

PREGNANCY AND LACTATION

Use in pregnancy:

- Considered to be the analgesic of choice in pregnant patients.
- Although it crosses the placenta, paracetamol is considered to be safe in normal therapeutic doses for short-term use as a minor analgesic/antipyretic in pregnancy.

Use in lactation:

- Excreted in breast milk.
- Maternal ingestion of paracetamol in normal therapeutic doses does not appear to present a risk to the nursing infant.

OVERDOSAGE

Liver damage is possible in adults who have taken 10 g or more of Paracetamol. Ingestion of 5g or more of Paracetamol may lead to liver damage if the patient has risk factors.

Risk Factors

If the patient:

- Is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St. John's Wort or other drugs that induce liver enzymes; or
- Regularly consumes ethanol in excess of recommended amounts; or
- Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia

Symptoms:

Toxic symptoms include vomiting, abdominal pain, hypotension, sweating, pallor, nausea, anorexia and abdominal pain.

The most serious adverse effect of an acute overdose of paracetamol is dose-dependent, potentially fatal hepatic necrosis. Clinical and laboratory evidence of hepatotoxicity may be delayed for up to one week. Major manifestations of liver failure such as jaundice, hypoglycemia and metabolic acidosis may take at least 3 days to develop. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, hemorrhage, hypoglycemia, cerebral edema and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Treatment:

In cases of overdose, methods of reducing the absorption of the ingested drugs are important. Despite the lack of significant early symptoms, patients should be referred to the hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Gastric lavage is essential even if several hours have elapsed. Prompt administration of 50g activated charcoal and 500 ml iced mannitol 20% by mouth, may reduce absorption. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentration is unreliable). If the history suggests that 15g Paracetamol or more has been ingested, administer one of the following antidotes:

i) Acetylcysteine 20% I.V.: Administer intravenously, 20% acetylcysteine immediately without waiting for a positive urine test or plasma level results: initial dose of 150mg/kg over 15 minutes, followed by continuous infusion of 50mg/kg in 500ml 5% glucose/dextrose over 4 hours and 100mg/kg in 1L 5% glucose/dextrose over 16 hours.

OR

ii) Oral Methionine: 2.5g immediately followed by three further doses of 2.5g at four hourly intervals. For a 3 year old child, 1g methionine every four hours for four doses has been used;

OR

iii) Oral Acetylcysteine 5%: 140mg/kg as a loading dose, then 70mg/kg every 4 hours for a total of 17 maintenance doses. If more than ten hours have elapsed since the overdose was taken, the antidote may be ineffective.

STORAGE CONDITION

Store below 30°C. Keep the container tightly closed. Protect from light and moisture.

SHELF LIFE

The product should not be used beyond the expiry date imprinted on the product packaging.

PACK SIZE

Available in a bottle of 60ml and 120ml suspension.

REGISTRATION NUMBER

MAL20046151XZ

KEEP MEDICINE OUT OF REACH OF CHILDREN JAUHI UBAT-UBATAN DARI KANAK-KANAK

For further information, please consult with a doctor or pharmacist.

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LT-230.03 Product Registration Holder And Manufacturer:
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